

Chapter 5: The Policy Pillar in Practice — CMMI Models, Waiver Strategy, and the Federal-State Policy Interface

How Healthcare Leaders Navigate the Federal Policy Landscape: APM Mechanics, 1115 Waivers, CMMI Model Selection, and the Regulatory Strategy That Determines Whether Payment Reform Succeeds or Stalls

From Policy Architecture to Policy Practice

Chapter 4 established Vermont's legislative reform architecture — the Acts, the mandates, the timelines, and the analytical foundation of the Oliver Wyman diagnosis. This chapter turns from architecture to practice: how do healthcare organizations, state agencies, and policymakers actually navigate the federal policy landscape that shapes every transformation decision?

The federal policy environment is not a monolithic force that organizations respond to passively. It is a complex, evolving, and frequently internally contradictory set of programs, rules, incentives, and administrative interpretations that skilled policy practitioners can navigate strategically. Understanding the CMMI model landscape, the mechanics of Medicaid section 1115 waivers, the structure of APM contracts, and the federal-state relationship that determines whether state-level reform efforts are supported or blocked by federal requirements is not academic knowledge — it is operational necessity for any organization or state agency working on healthcare transformation in 2026.

Section 1: The CMMI Landscape — What Exists, What Works, and What Has Been Discontinued

The Center for Medicare and Medicaid Innovation has been the primary federal vehicle for payment reform experimentation since its creation under the ACA. Over fifteen years, CMMI has launched more than fifty models, invested billions of dollars, and engaged thousands of healthcare organizations across the country. The results have been mixed in ways that carry specific lessons for organizations designing their own value-based care strategies.

The CMMI Track Record — An Honest Assessment

A 2023 independent evaluation of CMMI models found that the vast majority had not achieved statistically significant reductions in Medicare spending or improvements in quality. Most models were discontinued. The models that showed the most promise — ACO REACH (formerly Direct Contracting), AHEAD, the Making Care Primary model — share a set of characteristics that distinguish successful from unsuccessful CMMI designs.

Characteristic	What it means	Models with this characteristic	Models without it
Mandatory or near-mandatory participation	When high-cost, high-volume providers cannot opt out, the model's financial logic applies to the full population. Voluntary models allow opt-out by the actors who	AHEAD (for participating states, all hospitals must participate). Maryland All-Payer Model (mandatory for all Maryland hospitals). Vermont Act 68	Most CMMI ACO models. MSSP Track 1. Bundled Payments for Care Improvement (voluntary). Most demonstration projects.

	benefit most from the status quo.	(mandatory for all Vermont hospitals).	
Two-sided risk that is real	Financial downside for underperformance is sufficient to change organizational behavior. One-sided shared savings with minimal upside and no downside does not change hospital or health system decision-making at scale.	ACO REACH (mandatory downside risk). Global budgets (hospitals bear population health risk directly). AHEAD (state-level accountability with meaningful consequences).	MSSP Track 1 (upside only for most participants). Most early CMMI bundled payment models with modest downside. Primary Care First (limited financial exposure).
Adequate implementation support	Organizations need technical assistance, analytical tools, and sufficient runway to redesign care delivery before being held financially accountable for outcomes they cannot yet control.	AHEAD (multi-year pre-performance period with substantial cooperative agreement funds). Vermont's RHRC engagement. Making Care Primary (practice transformation support).	Many early CMMI models that launched with insufficient operational support and were discontinued when providers could not implement effectively.
Alignment with state regulatory environment	Federal models that conflict with or duplicate state regulatory structures create compliance complexity without adding value. Models designed in partnership with state agencies produce better results.	AHEAD (designed explicitly as federal-state partnership; requires state agreement and state regulatory alignment). Vermont-specific APM (state-federal co-design). Maryland TCOC (built on Maryland's existing HSCRC regulatory authority).	Federal models imposed on states without coordination with state Medicaid agencies, insurance regulators, or hospital budget review processes.

Figure 5.1 — CMMI model success factors: what distinguishes models that produce results from those that don't. Source: HTR analysis of CMMI evaluation literature; CMS model documentation.

The Current Active CMMI Model Landscape (2026)

As of 2026, the CMMI model portfolio has been significantly rationalized from its peak breadth under prior administrations. The models with the highest relevance to Vermont-style transformation are:

Model	Key features and relevance
AHEAD (Achieving Healthcare Efficiency through Accountable Design)	Six-state voluntary total cost of care model. Vermont in Cohort 2 (begins January 2027). Hospital global budgets for Medicare FFS. Enhanced primary care payments. \$150M/year additional Medicare funds for Vermont. The most relevant model for state-level transformation.
ACO REACH (Realizing Equity, Access, and Community Health)	Medicare FFS ACO model with mandatory two-sided risk. Full-risk capitation option. Equity focus including SDOH requirements. For organizations managing attributed Medicare populations outside of a state-level model, ACO REACH is the highest-accountability current federal ACO option.

Making Care Primary (MCP)	Multi-payer primary care transformation model in 8-state initial cohort. 10-year model. Three-stage progression from basic to advanced primary care capabilities. States not participating in AHEAD may find MCP a viable primary care transformation vehicle.
BALANCE Model	GLP-1 obesity drug coverage for Medicare and Medicaid enrollees at negotiated prices. Voluntary for manufacturers, states, and plans. Medicaid launch May 2026; Medicare Part D January 2027. Critical equity and chronic disease management model.
Bundled Payments for Care Improvement Advanced (BPCI-A)	Episode-based payment for acute and post-acute care episodes. Voluntary. For health systems and post-acute providers, BPCI-A remains a viable two-sided risk VBC entry point for specific service lines (joint replacement, cardiac, etc.).

Section 2: Section 1115 Waivers — The State's Primary Federal Policy Tool

Section 1115 of the Social Security Act gives CMS authority to waive certain Medicaid requirements and provide federal matching funds for experimental programs that promote program objectives. For states attempting comprehensive healthcare reform, the 1115 waiver is the most powerful federal policy tool available — enabling states to design Medicaid programs that would not be permitted under standard federal rules, to test new benefits and delivery models, and to demonstrate approaches that, if successful, can become national policy.

What 1115 Waivers Can and Cannot Do

Understanding the scope and limits of 1115 waiver authority is essential for any state or organization involved in Medicaid reform. The following framework organizes the most common 1115 waiver applications:

1115 Waiver Authorities: What States Can Accomplish

Coverage expansions beyond standard Medicaid categories — including populations not otherwise eligible, non-traditional benefit structures, and premium assistance programs that allow states to use Medicaid funds toward employer-sponsored insurance.

- Vermont's Global Commitment to Health demonstration extends coverage and benefits beyond standard Medicaid state plan requirements, enabling HRSN (health-related social needs) services, expanded developmental disability benefits, and substance use disorder treatment models not available under the standard state plan.

Delivery system and payment reform — enabling states to implement global budgets, ACO arrangements, and alternative payment models for Medicaid that are not permitted under standard Medicaid FFS or managed care rules.

- Vermont's alignment of Medicaid global budgets with AHEAD is executed through the Global Commitment to Health demonstration, which provides CMS authority to implement the Medicaid component of Vermont's hospital global budget design.

SDOH investments — since 2021, CMS has permitted states to use 1115 waiver funds for health-related social needs services, including housing supports, food assistance, and transportation, for Medicaid enrollees at risk of institutionalization or with complex social needs.

- Vermont's recent 1115 amendment approval authorizes HRSN services — enabling Blueprint's universal SDOH screening to connect to Medicaid-funded services, not just community resources.

Work requirement programs — subject to ongoing legal and policy uncertainty. H.R. 1 (2025) explicitly authorizes work requirements for Medicaid expansion enrollees, resolving the prior legal uncertainty in favor of state authority. Vermont has not implemented work requirements and has not signaled intent to do so.

The Vermont 1115 Waiver Portfolio

Vermont operates several 1115 demonstrations that together constitute its Medicaid transformation infrastructure. The Global Commitment to Health demonstration — Vermont's primary 1115 waiver — was originally approved in 2005 and has been renewed and amended repeatedly. The current authorization runs through December 2027, with the AHEAD alignment provisions approved in 2024. Vermont's AHEAD Medicaid global budgets for hospitals, beginning January 2026, operate under this demonstration authority.

Vermont's 1115 portfolio illustrates a principle worth stating explicitly: 1115 waivers are negotiated instruments. They are not applications that CMS approves or denies based on a checklist; they are the product of sustained federal-state negotiation in which the state advocates for the policy design it wants and CMS negotiates conditions, monitoring requirements, and budget neutrality standards that protect the federal government's financial interests. Vermont's ability to implement its current AHEAD-aligned Medicaid policy is the direct product of years of negotiation that produced a federal-state agreement on methodology, accountability, and withdrawal conditions. States that engage with CMS as partners in designing innovative demonstrations get better waiver terms than states that submit standard applications.

Budget Neutrality — The Binding Constraint

Every 1115 waiver is subject to a budget neutrality requirement: the federal government will not spend more under the demonstration than it would have spent under the standard program. Budget neutrality is calculated against a 'without-waiver' baseline — what Medicaid spending would have been if the state had not implemented the demonstration — and must be maintained over the demonstration period.

Budget neutrality is simultaneously the most important financial discipline in 1115 waiver design and the most technically complex. The without-waiver baseline is inherently uncertain — it requires projecting what would have happened counterfactually. States and CMS frequently negotiate over baseline methodology. Vermont's experience with AHEAD budget neutrality negotiations — where the methodology for calculating the Medicare global budget baseline was a specific condition of the state's AHEAD participation — illustrates that these technical negotiations have major financial consequences. Getting the baseline right (or wrong) can determine whether a waiver is financially sustainable for the state or produces unexpected federal financial pressure.

Section 3: Prior Authorization Reform — The Administrative Pillar of Policy Strategy

Prior authorization — the requirement that providers obtain health plan approval before delivering certain services — is one of the most significant drivers of administrative burden, care delay, and provider frustration in American healthcare. The average physician practice spends 14.9 hours per week on prior authorization tasks. Prior authorization denials delay care for 33% of patients and cause some to abandon treatment entirely. The administrative cost of prior authorization nationally is estimated in the tens of billions of dollars annually.

Prior authorization reform is a Policy pillar issue because it requires either federal rulemaking or state legislation to change meaningfully. Individual payers reforming their own prior authorization processes voluntarily have limited impact; the burden arises from the aggregate of PA requirements across dozens of payers, each with different criteria, forms, and approval timelines.

Federal Prior Authorization Reform: What H.R. 1 and CMS Rules Require

The Improving Seniors' Timely Access to Care Act, passed in 2022, required Medicare Advantage plans to implement electronic prior authorization, transparency reporting, and expedited review timelines. CMS implemented rules in 2024 requiring payers to respond to expedited PA requests within 72 hours and standard PA requests within 7 days — a significant improvement from previous norms where standard reviews could take weeks.

H.R. 1 (2025) included additional prior authorization reform provisions: public reporting of PA denial rates by service type, required disclosure of clinical criteria used in PA decisions, and stronger external appeal rights for denied requests. These are incremental improvements, not transformational changes. Full elimination of PA for evidence-based, guideline-concordant care — the policy that would most reduce administrative burden — remains aspirational.

Vermont's Prior Authorization Reform — Act 167's Provisions

Act 167 explicitly included prior authorization reform as one of its covered domains, directing AHS to develop simplified prior authorization processes and directing GNCB to report on prior authorization burden and denial rates. Vermont's Health Care Advocate has documented PA denial rates and challenged insurance rate filings that include excessive PA complexity costs. Act 68 builds on this by requiring insurers to demonstrate that administrative cost structures, including PA programs, are consistent with medical loss ratio requirements.

Oliver Wyman's community engagement findings are explicit about the real-world impact: providers described the CON process and prior authorization as major barriers to efficient care delivery, with cost-effective contracts unable to be secured in time due to administrative friction. Vermont's transformation agenda treats administrative simplification — including prior authorization reform — as an Operations pillar requirement that enables every other transformation initiative.

Section 4: The State-Federal Policy Interface — Navigating the Relationship

For state health agencies and the organizations that work with them, the federal-state policy relationship is simultaneously a resource and a constraint. Federal funding through Medicaid, CMMI models, HRSA grants, and the RHT Program provides the capital that makes transformation possible. Federal requirements — budget neutrality, CMMI model conditions, Medicaid state plan requirements — constrain the policy designs states can pursue. Federal policy uncertainty — created by administrative transitions, legislative changes like H.R. 1, and rulemaking processes that can reverse course — creates planning challenges for states with multi-year transformation timelines.

Managing Federal Policy Risk — Vermont's Approach

Vermont's AHEAD State Agreement exemplifies sophisticated federal policy risk management. The agreement includes specific conditions under which Vermont can unilaterally withdraw from AHEAD — before launch, with 30-day notice; after launch, with 6-month notice. The agreement includes conditions that protect GNCB's regulatory independence. It includes explicit provisions about methodology acceptance and dispute resolution. Vermont negotiated these withdrawal rights precisely because a nine-year commitment to a federal model involves significant policy risk — the model could change under a future administration, the methodology could produce unexpected financial outcomes, or federal policy changes could make AHEAD participation financially unsustainable.

Vermont's approach — engage the federal government as a partner, negotiate specific protections against adverse policy changes, maintain clear withdrawal rights, and design state-level statutory authority (Act 68) that does not depend on federal model continuation — is the right model for any state undertaking long-term federal-state policy commitments. States that design transformation strategies that are entirely dependent on continued federal program participation are strategically exposed; states that use federal models to accelerate and fund state-designed transformation that has independent statutory authority are more resilient.

The Policy Strategy Framework for Healthcare Organizations

For healthcare organizations navigating the policy environment — hospitals, health systems, payers, physician groups — the Policy pillar generates three distinct strategic

functions: regulatory compliance (understanding and meeting mandatory requirements), opportunity identification (finding federal and state programs that can fund or accelerate transformation goals), and advocacy (shaping policy in directions favorable to the organization's mission and the populations it serves).

Function	Key activities	Vermont-specific examples	HTR advisory service
Regulatory compliance	Monitor rulemaking; assess financial and operational impact of new requirements; develop compliance roadmaps; manage comment letter processes	Act 68 RBP compliance planning for FY2027; AHEAD hospital global budget participation; H.R. 1 Medicaid work requirement implementation timeline	Policy Strategy service line: Phase 1 Landscape and Exposure Assessment
Opportunity identification	Track CMMI model launches and eligibility; identify state waiver opportunities; evaluate RHT Program grant eligibility; assess AHEAD primary care participation options	Vermont hospitals: Act 68 transformation grants (\$2M); AHEAD EAST Fund; RHT Program subrecipient grants; BALANCE Model GLP-1 coverage	Policy Strategy service line: federal program opportunity assessment
Policy advocacy	Participate in public comment on proposed rules; engage state legislature on transformation legislation; provide testimony on hospital financial sustainability; participate in GMCB public meetings	Vermont hospital leaders: engagement with AHS transformation planning; community input on COE designations; participation in Health Care Delivery Advisory Committee public comment	Policy Strategy service line: Phase 3 Implementation Support — legislative testimony and comment letter drafting

Figure 5.2 — Policy strategy functions for healthcare organizations. Source: HTR Advisory framework.

Section 5: The Medicaid Policy Landscape — What H.R. 1 Changes and What It Does Not

The 2025 reconciliation law has changed the Medicaid landscape in ways that every healthcare organization and state agency must understand. The following framework distinguishes between provisions already in effect, provisions taking effect in late 2026 and 2027, and provisions whose implementation timeline remains uncertain.

H.R. 1 Medicaid provision	When it takes effect	Financial impact	Vermont-specific implications
Provider tax restrictions: ban on new or higher provider taxes in expansion states; phase-down of existing taxes	In effect for new taxes; phase-down beginning 2027	Reduces states' ability to finance their Medicaid matching share; intensifies pressure on state Medicaid budgets	Vermont's financing mechanisms will need review; state budget implications require modeling through 2031
State-directed payment caps at 100% of Medicare (expansion states)	2027	Reduces enhanced Medicaid payments that many states use to supplement provider rates above standard Medicaid	Vermont hospitals receiving state-directed supplemental payments face revenue reduction; GMCB global budget design must account for this

		levels; significant for safety-net hospitals	
Work and community engagement requirements (80 hours/month)	Late 2026 (states must implement by December 31, 2026)	10 million projected uninsured nationally by 2034; variable state impact depending on implementation; administrative burden on states and enrollees	Vermont has not expressed intent to implement work requirements; federal law now clearly authorizes them. Coverage implications for Vermont's expansion population require monitoring.
Eligibility redetermination every 6 months (vs. annual)	2026	Increases administrative burden; produces coverage churn as enrollees who remain eligible lose coverage due to administrative failures; disproportionate impact on working families	Vermont DVHA will face increased redetermination workload; outreach to ensure eligible Vermonters maintain coverage is an equity priority
Medicaid reimbursement reductions (DSH cuts, payment rate changes)	2026-2031 (phased)	\$911B over 10 years nationally; \$137B in rural areas; hospitals will experience revenue reductions	All 14 Vermont hospitals face reduced Medicaid revenue; most severe impact on CAHs with high Medicaid payer mix; integrated into hospital financial sustainability modeling

Figure 5.3 — H.R. 1 Medicaid provisions: timeline, financial impact, and Vermont implications. Sources: KFF analysis; CBO estimates; Pew Charitable Trusts (January 2026).

For Vermont specifically, the Medicaid revenue reductions create a financial stress that compounds the hospital financial crisis Oliver Wyman documented before H.R. 1 passed. Vermont's hospitals were already projecting 5-year cumulative deficits of \$700 million to \$2.4 billion under the pre-H.R. 1 baseline. The additional Medicaid revenue reductions accelerate the financial timeline for hospitals that were already financially distressed, and reduce the financial cushion for hospitals that were marginally sustainable. This is the fiscal environment in which Vermont's transformation must succeed — which makes the urgency of the transformation not just a policy preference but a fiscal survival requirement.

Section 6: Policy Strategy for Healthcare Leaders — The Practitioner Framework

Policy strategy for healthcare leaders is not primarily about lobbying or advocacy — though those functions matter. It is primarily about anticipation: understanding what the policy environment will require of organizations 18-36 months from now, positioning the organization to comply at manageable cost, and identifying the policy opportunities that can be converted into strategic advantage.

The Policy Monitoring Framework

Every healthcare organization needs a policy monitoring infrastructure proportionate to its size and regulatory exposure. The minimum viable policy monitoring framework includes four functions:

- **Federal rulemaking surveillance: Tracking proposed rules from CMS, ONC, and other relevant agencies through the Federal Register notice and comment process. Final rules typically take effect 60 days after publication, but proposed rules give 60-90 days of advance notice that organizations can use for impact assessment and comment letter preparation.**

- **State legislative monitoring:** Tracking legislation that affects the organization's operating environment through the state legislative session. Vermont's pattern of annual healthcare legislation — each session producing significant new requirements or opportunities — means that organizations not engaged in the legislative process are consistently surprised by its outputs.
- **CMMI model pipeline monitoring:** Tracking CMMI model announcements, application deadlines, and eligibility criteria. New CMMI models typically have narrow application windows; organizations that are not monitoring for them miss the opportunity to participate on favorable terms.
- **Regulatory intelligence:** Monitoring GMCB decisions, CMS guidance documents, and state agency program updates that affect payment and operating requirements. Vermont's GMCB publishes hospital budget guidance, RBP methodology updates, and program decisions that directly affect every Vermont hospital's financial planning.

The Policy Impact Assessment Framework

When a new policy is identified — a proposed rule, a new CMMI model, a state legislative development — the organization needs a structured impact assessment process. The HTR Policy Strategy service line uses a four-phase engagement structure (described in Chapter 16), but the core analytical logic can be applied internally:

- **Exposure mapping:** What specifically does this policy change require of our organization? What are the financial, operational, and compliance implications? What is the timeline?
- **Scenario analysis:** Under what range of outcomes (final rule differs from proposed rule, implementation delayed, model parameters change) does our strategic position change?
- **Response strategy:** What is the optimal response — compliance roadmap, comment letter participation, CMMI model application, advocacy engagement?
- **Stakeholder communication:** Who needs to know about this policy change, and what decisions does it require from the board, executive team, clinical leadership, and financial team?

Key Concepts

Term	Definition
Section 1115 waiver	Authority under the Social Security Act allowing CMS to waive standard Medicaid requirements and approve experimental programs that promote program objectives. Vermont's Global Commitment to Health demonstration is its primary 1115 vehicle.
Budget neutrality	The federal requirement that 1115 waiver spending not exceed what Medicaid spending would have been without the waiver, calculated against a negotiated without-waiver baseline.
CMMI (Center for Medicare and Medicaid Innovation)	Federal entity created by the ACA to test new payment and delivery models. Has launched 50+ models since 2010; AHEAD is the most relevant current model for state-level transformation.
Two-sided risk	A payment arrangement in which the organization shares in both savings (upside) and losses (downside) relative to a financial target. Mandatory downside risk changes organizational behavior more effectively than one-sided shared savings.

Prior authorization	Health plan requirement for pre-approval before delivering specified services. Oliver Wyman documented PA complexity as a major administrative burden for Vermont providers; Act 167 and Act 68 both address PA simplification.
Making Care Primary (MCP)	CMMI's 8-state multi-payer primary care transformation model. A potential vehicle for states not participating in AHEAD seeking federal support for primary care investment.
Federal Register	The official journal of the U.S. federal government, where CMS and other agencies publish proposed rules (open for public comment) and final rules (legally binding). Essential monitoring source for healthcare policy practitioners.

Sources: CMS CMMI model documentation; CMS 1115 waiver framework; KFF Medicaid policy analyses; H.R. 1 One Big Beautiful Bill Act (July 2025); Vermont Global Commitment to Health demonstration (CMS approval January 2025); Act 167 of 2022; Act 68 of 2025; AHEAD State Agreement (January 2025); American Medical Association prior authorization burden survey; Georgetown Center for Children and Families.